This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Qdenga (TAK-003) for the prevention of Dengue Fever

Patient Group Direction, version 008, issued 30 September 2026. Supersedes Version 007, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for dengue vaccination: Green Book chapter 15a (Dengue), NaTHNaC / TravelHealthPro dengue factsheet and the Qdenga SmPC. There is no NICE or NICE CKS topic on dengue.

Overview

Dengue fever is caused by the dengue virus, of which there are four serotypes (DENV-1, DENV-2, DENV-3, DENV-4). The disease is transmitted by Aedes mosquitoes and is endemic in tropical and subtropical regions worldwide, affecting over 400 million people annually.

Assessment

- **Travel destination risk:** Assess if the patient is travelling to or will reside in dengue-endemic areas.
- **Previous dengue infection:** Obtain history of previous dengue infection, if known.
- **Duration of stay:** Evaluate the intended duration and timing of travel.
- **Immunisation status:** Confirm current vaccinations and previous dengue serology if available.

Management

- **Vaccination schedule:** Two doses of Qdenga administered 3 months apart via subcutaneous injection.
- **Mosquito bite prevention:** Advise on insect repellents, protective clothing, and accommodation with air conditioning and/or screens.
- **Timing:** First dose should be administered at least 3 months prior to travel when possible.

Red Flags and Contraindications

- **Immunocompromised individuals:** Live attenuated vaccine is contraindicated in those with severe immunocompromise.
- **Pregnancy and breastfeeding:** Vaccine is not recommended during pregnancy or breastfeeding.
- **Severe acute illness:** Defer vaccination if the patient has acute febrile illness or other significant illness.
- **Concurrent live vaccines:** If other live vaccines are required, Qdenga should be given simultaneously or separated by at least 4 weeks.

Patient Group Direction

for the supply/administration of

Qdenga (TAK-003)

for the prevention of

Dengue Fever

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent

	experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Immunisation against dengue fever in adults aged 18 years and over travelling to or residing in dengue-endemic areas.
Inclusion criteria	Age 18 years or over Planning travel to or residence in dengue-endemic area(s) Willing to receive two doses, 3 months apart Able to provide informed consent Not pregnant and not breastfeeding No immune deficiency of any cause (see exclusion criteria)
Exclusion criteria	Age under 18 years Pregnancy or breastfeeding Known hypersensitivity to any component of the vaccine Any congenital or acquired immune deficiency, including: immunosuppressive therapy such as chemotherapy, or systemic corticosteroids at 20 mg/day prednisolone (or 2 mg/kg/day) or more for 2 weeks or longer, within the previous 4 weeks; active malignancy; symptomatic HIV infection, or asymptomatic HIV infection with evidence of impaired immune function (SmPC 4.3) Acute fever or significant intercurrent illness Planned administration of another live vaccine within 4 weeks before or after Qdenga History of Guillain-Barré syndrome following prior dengue

	vaccination
Cautions	Immune deficiency of any degree excludes (see exclusion criteria); there is no immunocompromised group in whom Qdenga is given under this PGD Use with caution in patients on anticoagulant therapy (assess bleeding risk) Patients should avoid pregnancy for at least 4 weeks after each dose Consider serological testing in those with previous dengue infection Administration should be deferred in patients with acute febrile illness
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Qdenga (TAK-003), powder and solvent for solution for injection: a vial of lyophilised vaccine reconstituted with the 0.5 mL solvent in the pre-filled syringe supplied. Reconstitute immediately before use.
Legal category	POM (Prescription Only Medicine)
Storage	Store in a refrigerator between 2°C and 8°C. Protect from light. Do not freeze. After reconstitution, use immediately or within 30 minutes at room temperature (SmPC).
Route/method of administration	Subcutaneous injection, preferably in the deltoid area of the upper arm
Dose and frequency	First dose: 0.5 mL subcutaneously Second dose: 0.5 mL subcutaneously, administered 3 months after the first dose
Quantity to be administered and/or supplied	2 doses of 0.5 mL each over a 3-month period
Maximum or minimum treatment period	Two doses 3 months apart complete the course. No booster dose is authorised under this PGD; the need for a booster has not been established (SmPC).
Adverse effects	Very common (≥1/10): Injection site pain, headache, myalgia Common (≥1/100 to <1/10): Malaise, fatigue, fever (usually within 7 days of vaccination)

	Uncommon (≥1/1,000 to <1/100): Rash, pruritus, arthralgia Rare: Guillain-Barré syndrome (very rare, temporal relationship to vaccination unclear) Report all suspected adverse effects via the Yellow Card scheme at https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of administration, dose, form, route and anatomical site, quantity administered, batch number and expiry date • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and the actions taken, and that the vaccine was administered under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with Qdenga. Ensure the patient understands the vaccine schedule, storage requirements, and what to expect after vaccination.
Follow-up advice to be given to patient or carer	Attend for second dose appointment in 3 months Report any unusual or severe symptoms to your healthcare provider or via the Yellow Card scheme Seek medical advice if fever develops or persists beyond 7 days after vaccination Avoid pregnancy for at least 4 weeks after each dose if of childbearing potential

	Continue mosquito bite prevention measures (insect repellent, protective clothing) even after vaccination Seek immediate medical attention if you develop rash, difficulty breathing, or signs of anaphylaxis after vaccination If you experience signs of dengue fever (fever, headache, rash, joint pain) whilst travelling, seek medical attention promptly Keep a record of your vaccination dates and bring documentation when travelling

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 15a Dengue; NaTHNaC TravelHealthPro, Dengue factsheet; Qdenga Summary of Product Characteristics, current version
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Records: traceability

RECORD THE VACCINE NAME AND BATCH NUMBER for every dose administered. This is a requirement for any biological product and it is what makes a recall actionable.

Record also the expiry date, the dose, the route, the anatomical site, and where more than one vaccine was given at the same visit, the site of each.

Record the name and registration number of the person administering, and that the vaccine was given under this PGD.

Consent in children and young people

This PGD is restricted to adults aged 18 years and over, so consent from a person with parental responsibility or on the basis of Gillick competence does not arise. Anyone under 18 is excluded; record the advice given.

Version and change record

Version: v008

Issued: 30 September 2026

Supersedes: Version 007, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v007

Issued: 14 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The title and cover now say the vaccine is for the prevention of dengue fever, not 'the treatment of Dengue Fever Prevention'.

The maximum treatment period row no longer mentions an optional booster, which had no dose row, no inclusion criterion and no SmPC basis; two doses 3 months apart complete the course and no booster is authorised.

The third repetition of the live vaccine immune deficiency statement in the cautions row was removed; the exclusion and the first caution remain.

The records row has its punctuation restored and now asks for the anatomical site, batch number and expiry date.

The consent in children and young people block now states that it does not apply, because this PGD excludes anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references are attributed to the Green Book, NaTHNaC and the SmPC. There is no NICE CKS topic on dengue; the previous version cited one.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Observation after vaccination, Cold chain and excursions, Disposal, Records: traceability, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Exclusion criteria: the previous version excluded severe immunocompromise only and permitted mild to moderate immunosuppression under a caution. The Qdenga SmPC (4.3) contraindicates any congenital or acquired immune deficiency, immunosuppressive therapy including systemic corticosteroids at 20 mg/day prednisolone or more for 2 weeks or more within the previous 4 weeks, and symptomatic HIV or asymptomatic HIV with impaired immune function. The exclusion now says that; the caution is gone.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 10 September 2026

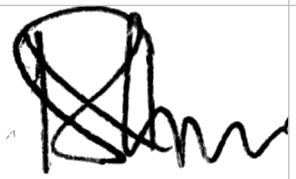
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Name and form corrected to the SmPC: Qdenga is a powder and solvent for solution for injection, reconstituted before use. The previous row described a ready-to-use suspension and then referred to reconstitution.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.